

Benchmark Personal Supervision Model

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Abstract

Clinical supervision of counselors in training (CITs) is essential in the academic journey for clinical mental health counselors. The objective learning elements are only a portion of what is needed to strengthen the skills and identity of the CIT. CITs must train in the practical qualities of clinical counseling. Supervision is the arena in which CITs are prepared for these elemental practical essentials. This paper will discuss the author's theoretical concept, preferred developmental approach, and process model for developing the CIT's relational competencies integrated together and with a biblical worldview.

Keywords: attachment theory, psychodynamic supervision, longbill, hardy, & delworth model, systems theory

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Knowing firsthand the effects of a healthy relationship on human development, the author can attest to the necessity of appropriate and transformational bonds built between supervisor and supervisee (or counselors in training; CITs). Todd Hall (2021, p. 71) empirically supports this concept in developing humans and adults. Research proves that humans need other humans and that the human brain depends on relationships to grow (Hall, 2021, p. 71). In his 2010 book, *The Mindful Therapist*, Siegel discusses the fundamental need of the human brain to feel seen, soothed, safe, and secure to shift from limbic processing to executive processing. In 2020, Baier et al. discovered that as much as 70% of therapeutic outcomes can be attributed to an attuned alliance between counselor and client. The author concludes that the relational aspects of a clinical supervisory model are equally, if not superiorly, crucial to the development of CITs as the skills acquisition and other objective elements. Therefore, he has chosen a theory, developmental approach, and process model that aligns with his worldview and belief that relationship is paramount in supervision.

Foundational Theoretical Model

A relational supervision model necessitates a relational theory of change and human psychology. A cursory review of the body of literature concerning theoretical models of supervision reveals the Psychodynamic theory to be a highly relational concept. In fact, Norberg et al. (2016) emphasize that emerging theorists using a relational model of psychodynamic supervision place the relationship at the center of supervision. When the relationship is at the center of the process, it is less likely that the supervisor is treating the CIT as a paycheck or as a checklist. Though the author agrees that there are times when supervisors must assess and evaluate (and be assessed and evaluated), and there are times when skills acquisition is the focus

of a supervision session, these would-be mechanical practices can grow in transformational power as the alliance between supervisor and CIT is strengthened.

According to Bernard and Goodyear (2019, p. 22), the psychodynamic model has four categories emphasized through the supervision process. These categories are relationship, sense of self, assessment and diagnosis, and interventions. Bernard and Goodyear contend that the key to psychodynamic supervision is that all these categories promote the therapeutic alliance. The relationship, for instance, is seen as “the crucible of psychotherapeutic change” (Bernard & Goodyear, 2019, p. 22). Meaning that CITs do not develop skills outside of this “crucible.” In keeping with the developmental and process models (discussed in detail below), the sense of self can best be developed in CITs as a supervisor aptly and interpersonally brings the emerging counselor into contact with themselves. Often, new counselors are unaware of their thoughts, feelings, emotions, and cognitive blocks as they work with clients. As the supervisor watches these dynamics unfold and brings the CIT into an awareness of herself, she can build the sense of self central to psychotherapeutic change. Being aetiologically a medical model of psychology, it makes sense that assessment and diagnosis remain the framework for operationalizing the psychodynamic model of supervision. As with all supervision, organizing the relationship in a fashion conducive to measuring and observing change is essential to empirical outcomes (Bernard & Goodyear, 2019, p. 142). This is the author’s least favorite counseling category and of this model. However, he admits that the facts of observable change are more authoritative than anecdotal stories and, therefore, hold more power for healing dysregulated and disturbed humans. With every theory, there will be agreements and disagreements, and the scales tip in favor of a psychodynamic theory of change through relationships despite the medical ancestry of this model.

Conversely, the intervention category is likely the author's reason for choosing this model from which to build. As will be evident from the following two sections, the author favors interventions that have at their core the desire to provide a safe haven and a secure base. The psychodynamic model allows the author to select the most desirable interventions and maintain a central focus on the interpersonal dynamics of the supervisory relationship.

In recent years, improvements have been made to the psychodynamic theory of supervision. One such improvement takes the "crucible" of the theory seriously, as stated that a relational supervisor sees their authority differently than a traditional psychodynamic supervisor. The relational supervisor uses a less authoritative construct as an observer and fellow student to emphasize the process with the CIT over the content of theory or skills acquisition (Norberg et al., 2016). This fits well with the author's temperament in counseling relationships. The author utilizes a "one down" approach to counseling to promote agency and independence in clients. It is fitting that a less directive and authoritative approach to supervision would allow CITs to develop a sense of self congruent with the need to sift through a thousand thoughts a minute as they sit with clients and to conceptualize a multitude of dispositions and goals daily. Once more, providing a shame-free space for awareness and processing is elemental to the author's supervision model. The psychodynamic theory provides a foundation for developing CITs through encouragement and support.

Developmental Approach

Encouragement and support are the most widely used interventions in the Loganbill, Hardy, and Delworth supervision model (Bernard & Goodyear, 2019, p. 33). It becomes necessary then for a supervisor to stay aware of his work when using this model. As transformational as encouragement and support can be, there is still room for challenge, paradox,

and boundaries within the supervision relationship. However, the Loganbill et al. model is still effective and adaptable to the author's integrated model. The three stages of the model are as good as any to identify how CITs are becoming self-aware, flexible, and comfortable with ambiguity (Tannen et al., 2019). Ambiguity is something the author has been bewildered by with past supervisees. Many who are capable academicians do not grasp the concept of the meandering, circuitous river clients invite us to travel with them. Using this model and discerning between the stagnation, confusion, and integration stages provides a structure to scaffold skills and dispositions throughout supervision.

However, these stages should be seen as something other than a linear progression. More appropriately, they are a growth cycle through experiences with the supervisor and clients. For example, a CIT can be in the stagnation stage early on or later in development. The supervisor must assess if the CIT is experiencing a lack of awareness (early CITs) or a sense of inability to deepen the therapeutic process (later CITs) (Bernard & Goodyear, 2019, p. 32). Thus, the supervisor must be aware of the eight issues CITs face in development. Following and assessing these eight issues makes the model complicated and sometimes difficult to follow. However, the author still prefers this model due to the ability to integrate it into a relational and systems approach.

The other two stages of the model (confusion and Integration) can be seen as the growth-promoting stages of the model. Working with and not just describing these stages catapults a CIT from one level of development to another. Speaking of descriptive, Tannen et al. (2019) conclude that one drawback to this type of model (phase model) is its descriptive rather than process nature. However, the author contends that every developmental model must have some aspect of description to it. When assessing and communicating areas of strength and concern to a

CIT, it is helpful to have language through which to do so. Descriptions can either hinder or help our concept of supervision. Similarly, one can see that a supervision session focused strictly on the process would leave a CIT wondering what just happened. Describing the goals and dispositions of development provides the CIT with an understanding of the process.

One drawback the author notes is the need for more description of linear progression. Cycling and recycling through the stages can be monotonous and discouraging. As a supervisor, one would desire to show a CIT where and how they have grown. Conversely, walking a CIT through these stages will help them have the tools they need to see how they are recycling through them as they continue to develop post-supervision.

This model is still a good candidate for integration with a systems process model, the psychodynamic theory of supervision, and a biblical worldview because of the way it lends itself to interpersonal processing and attachment-focused relationships. To explore this integrated model further, we turn to a systems process model.

Supervision Process Model

As in all of life, we do not exist in a vacuum in clinical supervision. There is a system, and in fact, multiple systems, interacting within the supervision process. These systems operating through clinical supervision, including supervisor/supervisee, supervisee/client, supervisee/self, and supervisor/self, all play a role in how effective supervision relationships are (Barnard & Goodyear, 2019). That is to say, individuals are not the most effective focus of a supervision model. Suppose we focus only on individual CITs' development, or only on supervisors' skills and dispositions, or only on administrative tasks. In that case, we are missing the bigger picture of overt and covert interpersonal interaction.

This model integrates with the author's supervision model as the relationship is also the center of

this concept. Holloway describes the essential factors of these systems as relationships, supervisor and supervisee, contextual dimensions, and process dimensions (Barnard & Goodyear, 2019, p. 53). Continuing this thought, this supervision model is seen as mutually influential and interrelated parts of a dynamic process between the six dimensions (supervisor, organization, client, supervisee, supervisor functions, and learning tasks) (Holloway, 2016).

As a supervisor works to recognize the interpersonal aspects of the relationship overtly, the CIT can begin to overlay this experience with clients, better understanding their role in the healing clients are seeking. This “system” of relationships at play can generally describe the model the author is developing for himself. When a relational dynamic such as the ability to soothe another person’s dysregulated brain occurs in supervision, the CIT experiences this for themselves; the supervisor explains the phenomenon and helps the CIT transfer this experience to clients, then the development of CITs is both objective and process-oriented. Yet, how a supervisor and supervisee affect one another is an additional vital system to consider.

In this system, the background, culture, aetiology, theoretical perspectives, and other factors influence how effective the relationship is. Ignoring this system at play between and in-between supervisor and CIT could prove a disastrous result in attunement and alliance, distracting from or derailing the process. Similarly, the contextual dimensions of the supervision relationship form their own system depending on the organization, administration, and clientele in operation (Barnard & Goodyear, 2019, p. 54). One cannot rule out the interplay of these factors, but one must be aware of and control for the effects on the outcome of supervision where possible. As well as contextual factors, one must plan for and work with process factors in this model. Understanding the undercurrents of the supervisor’s functions and the CIT’s learning tasks to bring about the outcome of supervision is another system to conceptualize. Recognizing

the supportive role versus the distracting role this aspect of the system (or sub-system itself) can play is essential to understanding Cit's behavior, supervisor behavior, and progress or delays in skills and disposition acquisition.

This model is appealing because the relationship is at the center of all six factors of the model (Holloway, 2016). No matter what part of the system you focus on, Holloway continues to bring the focus back to the center. Future research into how the elements of counseling supervision pose a developmental interplay as the relational process is carried out will be necessary (Holloway, 2016). For now, we will turn to the final piece of the integration puzzle: one's worldview.

Biblical Worldview

Not all supervision models will easily integrate with one's worldview. It is impossible to consider all models and their integration into all worldviews in this context. Therefore, we will consider the above-mentioned models in association with a biblical worldview.

What is common in all of the models above? And what would tie these supervision models to a biblical worldview? Relationships! We are first introduced to biblical relationships in Genesis 1:27, where God the Father himself shows us the relational nature of the triune God (Father, Son, and Holy Spirit) (*NIV*, 1973/1984). It is evident that not one independent being of this three-person God created, named, envisioned, planned, or prepared the world, and all this is in it, including his crowning masterpiece of humans. God is relational. He created humans to be in relationship; male and female he created them (Genesis 1:27, *NIV*). He created us in this fashion both to better reflect his own image and to highlight the need for other human beings.

Because of this, I believe Fitch et al., 2010, outline what would be a more biblical model: the Attachment-Caregiving model of supervision. Through this model, relationship, the attachment theory, and humankind's createdness take center stage.

This model calls for supervisors to become a safe haven that provides safety enough for CITs to deactivate the attachment system and activate the exploratory system. The CIT can then cognitively process becoming a good counselor (Fitch et al., 2010). More biblically speaking, through Psalm 23, a supervisor can help the CIT see God as a safe haven to co-regulate with or to look to for soothing their dysregulated system. Psalm 23 speaks of God as one who provides, refreshes, restores, leads, is with, reassures, feeds, fills, pursues, and shelters (*NIV*). This is a safe haven and more. Even if a CIT is not a follower of Christ and does not want to see God in this way or process spirituality through a biblical lens, a supervisor can soothe with God through this understanding and provide the grounding and safety a CIT needs.

Once a safe haven has been established (this is similar to a therapeutic alliance discussed above), a secure base can be established. Being regulated long enough, the supervisor's exploratory system provides continued guidance and safety but allows the CIT to navigate threats autonomously (Fitch et al., 2010). This understanding leads to the conclusion that, as Jesus said, God will never leave us alone (Hebrews 13:5, *NIV*). When a CIT or a supervisor can regulate their system, they can process the systems and interpersonal dynamics of counseling.

As discussed above and in keeping with an autonomous model, when supervisors empower CITs to grow and develop into competent counselors, the results are strong, wise counselors. Using an attachment-caregiving model is part of a holistic approach to the development of CITs. Creating safety and security no matter what the CIT's background is proven to bring autonomy and confidence to CITs (Mammen, 2020).

This mimics the natural way Jesus taught the disciples (Mark 6:7-13) and the way God modeled relationships (James 4:8; Jeremiah 32:38, *NIV*). Jesus first asked the disciples to accompany and witness him interact with people. Then, he asked them to go out and do what he did. After these encounters, he brought the disciples back for processing. Eventually leaving them to an autonomous ministry that is still connected to the Godhead. God never entirely leaves us alone but gives us the skills and character we need to fulfill his calling on our lives. Supervision is no different. God is with us to soothe us, show us how to love and develop people and give us the grounding and footing we need to embody healing relationships.

Conclusion

The supervision model proposed in this paper is an integration of psychodynamic theory (focusing on relationship as the “crucible”), a developmental model that uses interpersonal processes to help CITs develop self-awareness, flexibility, and comfortability with ambiguity, a process model that allows for supervisors to delineate the multitude of factors holding sway on a CITs growth, and a worldview that envisions a God who is the author of safety and security in relationships. Folding this together like a good cake, one can see that functions, skills, organizations, and metrics are all critical in a supervision model. However, relationships being relationships, if the focus is not on how a supervisor and CIT are bonding and making sense of therapy together, we have vacuumed out the soul of counseling.

References

- Baier, A. L., Kline, A. C., & Feeny, N. C. (2020). Therapeutic alliance as a mediator of change: A systematic review and evaluation of research. *Clinical Psychology Review, 82*.
<https://doi.org/10.1016/j.cpr.2020.101921>
- Bernard, J. M., & Goodyear, R. K. (2019). *Fundamentals of clinical supervision* (6th ed.). Allyn & Bacon.
- Fitch, J., Pistole, M. C., & Gunn, J. (2010). The bonds of development: an Attachment-caregiving model of supervision. *Clinical Supervisor, 29*(1), 20–34.
<https://doi.org/10.1080/07325221003730319>
- Hall, T. W. (with Hall, M. E. L.). (2021). *Relational spirituality: A psychological-theological paradigm for transformation*. InterVarsity Press.
- Holloway, E. L. (2016). *Supervision essentials for a systems approach to supervision*. American Psychological Association. <https://doi.org/10.1037/14942-002>
- Mammen, M. A. (2020). Attachment dynamics in the supervisory relationship: Becoming your own good supervisor. *Journal of Psychotherapy Integration, 30*(1), 93–101.
<https://doi.org/10.1037/int0000164>
- Norberg, J., Axelsson, H., Barkman, N., Hamrin, M., & Carlsson, J. (2016). What psychodynamic supervisors say about supervision: Freedom within limits. *Clinical Supervisor, 35*(2), 268–286. <https://doi.org/10.1080/07325223.2016.1219896>
- Siegel, D. J. (2010). *The mindful therapist: A clinician's guide to mindsight and neural integration*. Norton.

Tannen, T., Daniels, M. H., & Koro-Ljungberg, M. (2019). Choosing to be present with clients: an evidence – based model for building trainees’ counselling competence. *British Journal of Guidance & Counselling*, 47(4), 405–419.

<https://doi.org/10.1080/03069885.2017.1370694>

The Holy Bible, New International Version. (1984). International Bible Society. (Original work published 1973).